

Who Pays After Medicaid Expansion?

Coverage, spending and utilisation across payers in US states, 2010–2018

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Setting and question

In 2014 the Affordable Care Act offered states the option of expanding Medicaid to adults under 138% of the federal poverty line, and opened the Marketplace for subsidised private insurance. Medicaid spending rose in expansion states with the new enrollment. Whether it displaced spending by other payers is less understood. If expansion moved people out of private plans, or shifted the cost of care they already received, private and Medicare spending should fall in expansion states relative to states that did not expand. We test that hypothesis on coverage, spending and encounters for all three payers.

Data and design

IHME Disease Expenditure Project (DEX 2.0): state-level spending and encounters by payer and setting, all ages, 2010–2018, fit to SHEA state expenditure. Uninsurance from CPS. **25 states** expanded on 1 January 2014; the **19** that never expanded serve as controls. Later expanding cohorts are dropped ($n \leq 3$ per year).

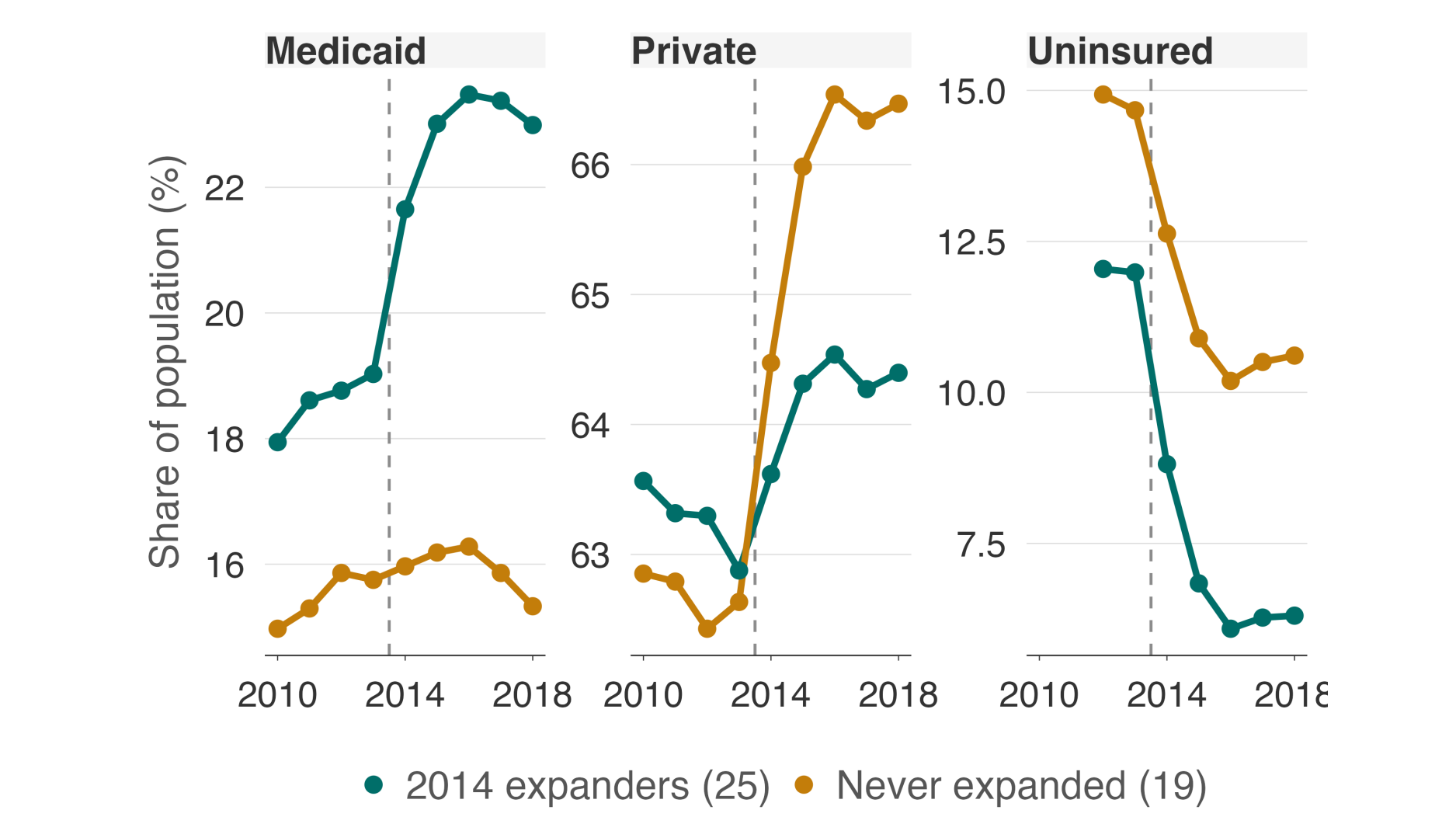
Callaway and Sant’Anna (2021) group-time difference-in-differences against the never-expanded states, conditioning on covariates X : poverty rate, median household income, pre-expansion uninsurance, and the shares under 18 and over 65:

$$\Delta Y_t = Y_t - Y_{2013}$$

$$ATT(t) = \mathbb{E}[\Delta Y_t \mid D = 1] - \mathbb{E}[\mathbb{E}[\Delta Y_t \mid X, D = 0] \mid D = 1]$$

Figures plot $ATT(t)$; tables report its 2014–2018 average. **Inference** is a multiplier bootstrap, 1,000 draws, clustered on state. **Parallel trends** is a Wald test of $ATT(t) = 0$ across the three pre-expansion placebo years, χ^2_3 on the analytic clustered variance. **Multiplicity** is Benjamini–Hochberg across the 66 estimates in panel 4.

1. Expansion covered the uninsured



Coverage	Effect	95% CI	Pre-test
Medicaid	+3.69	[+2.27, +5.12]	0.84
Medicare	-0.20	[-0.54, +0.14]	0.72
Uninsured	-1.38	[-2.48, -0.28]	0.30
Private	-2.16	[-3.07, -1.24]	0.96

Medicaid coverage rose **3.7 points** more in expansion states, and the uninsured rate fell a further **1.4 points**. Private coverage grew in both groups, 62.9 to 64.4% in expanders and 62.6 to 66.5% in controls, so **-2.2** is controls gaining faster, not expanders losing: Marketplace subsidies reach 100 to 138% FPL where Medicaid does not.

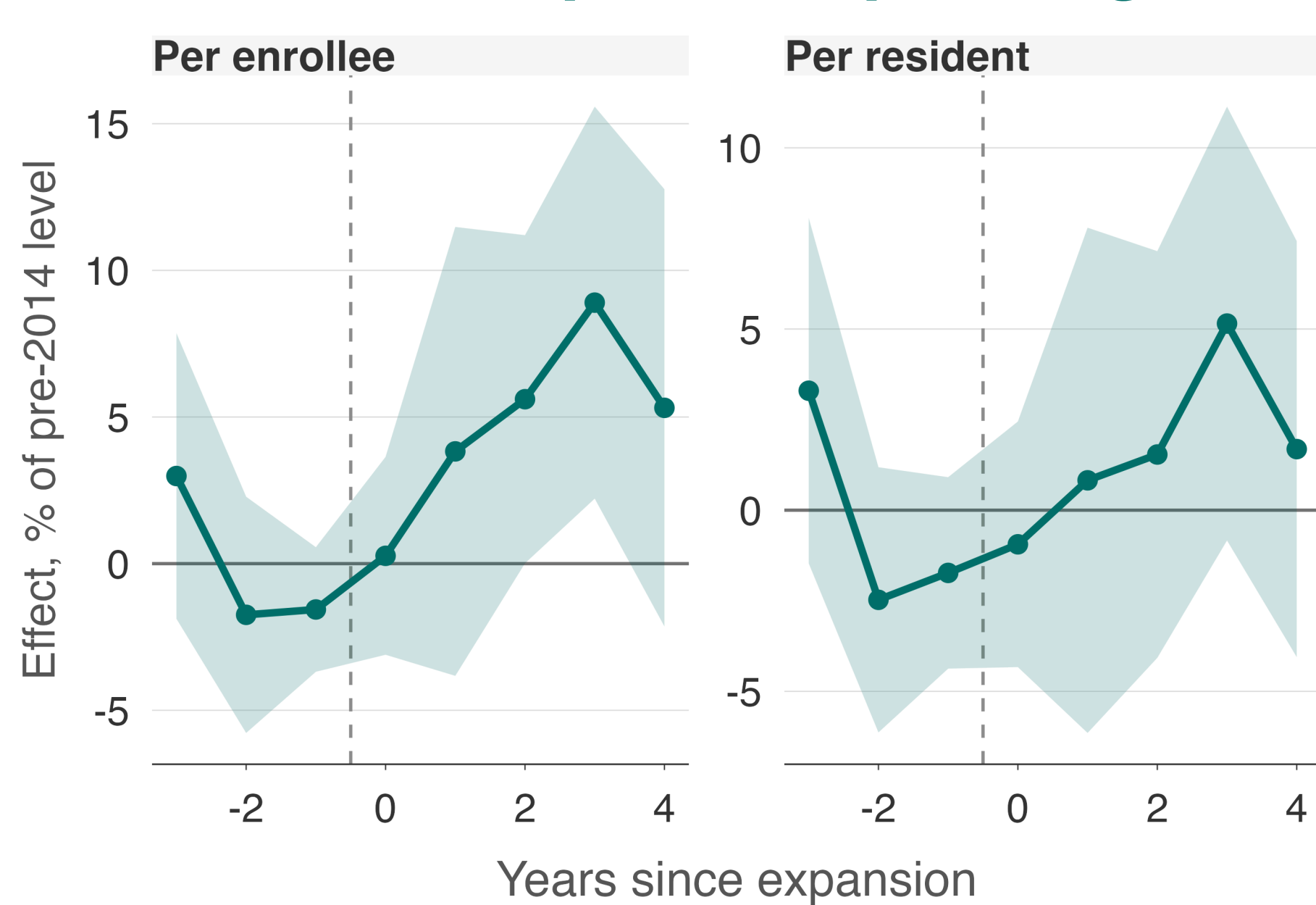
2. New Medicaid enrollees had similar healthcare consumption



Difference-in-differences effect at each year relative to 2014, as a percentage of the pre-expansion level in expansion states. 95% intervals from multiplier bootstrap.

Spending across expansion states rose **18.0%** (SE 3.9) after expansion alongside **19.9%** increased enrollment, an additional **\$189 per resident per year** on a pre-expansion base of \$1,051. Spending per enrollee was not affected: **-3.3%** (SE 4.4), so the states that expanded bought more of the same care rather than more expensive care.

3. No decline in private spending



Private spending per enrollee follows an increasing but insignificant path across 2014–2018, averaging **+4.8%** (SE 2.8) above the pre-expansion level; per resident the average is **+1.7%** (SE 2.4). Neither margin shows the reduction that crowd-out would produce.

Encounters per enrollee are positive in all five settings, **+2.2 to +5.5%**, and significant in none, so what movement there is looks like use rather than price.

The interval on spending per resident, **[-3.0, +6.3]%**, excludes a fall larger than 3% of the pre-expansion level, or about **\$80 per resident per year**. That is the tightest bound the design supports against crowd-out.

4. No effect outside Medicaid

We find no significant effect of the 2014 expansion on spending or encounters, per resident or per enrollee, in any setting, for any payer other than Medicaid. Of the **66 estimates** below, four survive correction for multiple comparisons, and all four are Medicaid on the per-resident margin. Among the **44 non-Medicaid** estimates, one clears 5% before correction and none survives it.

Precision is uneven. On the aggregate margins the intervals bound the effect: Medicare spending per resident is **+0.4%** [-2.3, +3.0] and private **+1.7%** [-3.0, +6.3], ruling out displacement larger than 3% of either payer’s pre-expansion spending. In the nursing and pharmacy cells they span 14 to 21 points, where the statement is insufficient evidence rather than no effect.

Spending per resident

Setting	Medicaid	Medicare	Private
All care	+18.0 (3.9)*	+0.4 (1.3)	+1.7 (2.4)
Ambulatory	+23.6 (5.8)*	-0.9 (1.2)	+2.1 (2.4)
Emergency	+21.7 (8.1)	+2.9 (4.9)	-0.2 (1.9)
Inpatient	+22.5 (4.5)*	+0.5 (1.4)	+1.2 (3.4)
Nursing	+2.0 (6.9)	+3.0 (5.6)	+4.6 (7.2)
Pharmacy	+11.1 (12.8)	+0.8 (2.9)	+0.8 (5.2)

Spending per enrollee

Setting	Medicaid	Medicare	Private
All care	-3.3 (4.4)	+1.5 (0.8)	+4.8 (2.8)
Ambulatory	+0.0 (6.2)	+0.9 (0.8)	+5.3 (3.0)
Emergency	-2.0 (7.8)	+3.8 (2.7)	+3.2 (2.2)
Inpatient	+4.4 (4.9)	+0.8 (0.9)	+4.0 (3.6)
Nursing	-14.4 (9.7)	+4.1 (4.9)	+7.3 (7.3)
Pharmacy	-22.5 (18.0)	+3.2 (1.4)	+4.2 (4.7)

Encounters per resident

Setting	Medicaid	Medicare	Private
Ambulatory	+15.5 (7.7)	-0.2 (1.3)	+1.3 (2.9)
Emergency	+7.7 (6.2)	+0.3 (3.1)	+0.6 (3.7)
Inpatient	+21.2 (6.0)*	+0.1 (1.2)	+0.7 (3.3)
Nursing	+4.5 (7.4)	+1.5 (4.7)	+2.7 (6.2)
Pharmacy	+0.4 (15.8)	-0.7 (2.4)	-0.8 (3.6)

Encounters per enrollee

Setting	Medicaid	Medicare	Private
Ambulatory	-4.4 (9.2)	+1.1 (1.0)	+4.4 (3.0)
Emergency	-5.5 (6.4)	+2.1 (1.6)	+3.8 (3.3)
Inpatient	+3.4 (7.0)	+1.5 (0.9)	+3.1 (3.3)
Nursing	-10.4 (10.0)	+3.2 (4.4)	+5.5 (6.4)
Pharmacy	-37.8 (23.1)	+2.1 (1.2)	+2.2 (3.1)

Effect as % of the pre-2014 level (SE). * survives Benjamini–Hochberg correction across the 66 estimates. Encounters are not summed across settings: a prescription fill and a hospital stay are not the same unit.

Limitations

Estimates apply to the 2014 cohort and the never-expanded states; the seven later expanding cohorts are excluded because at one to three states each, the estimator’s variance is not identified.

Out-of-pocket spending is excluded. DEX combines cost-sharing by the insured with all spending by the uninsured, and expansion moves that mix.

Coverage is a state-level share, so we see that private coverage grew more slowly in expansion states, not which individuals switched.

Null results are bounded, not confirmed. We report intervals rather than equivalence tests, so the claim is that effects larger than those bounds are excluded.

Expansion’s new enrollees cost about as much as the old ones, and private insurance did not shrink to pay for them.

Medicaid spending per resident rose 18%; per enrollee it did not move. Private coverage grew in expansion states too, just more slowly than in states where the Marketplace covered the gap.